

HEALTHCONNECT EXPERIENCE LAB

Final Analytics Executive Summary | Week 8

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Business problem and analytical scope

HealthConnect seeks to improve appointment attendance and target administrative support where it could be most useful. Of 5,000 recorded appointments, 4,737 have an observable attendance outcome; 263 cancelled appointments are excluded from the No-Show Rate denominator. Within the attendance-observable population, 2,423 appointments were recorded as no-shows, yielding an Overall No-Show Rate of 51.15%.

Validated findings

Booking Lead Time is the strongest validated standalone differentiator among the factors examined. The No-Show Rate was 39.13% for appointments booked 0–30 days in advance, compared with 63.95% for those booked 31–60 days in advance: an observed gap of 24.82 percentage points. Recorded Previous No-Shows adds complementary group-level information: 46.30% for appointments with none recorded versus 57.83% for those with at least one, a gap of 11.53 percentage points.

Decision implication

These findings support prioritising longer-lead appointments for further evaluation of attendance-support measures. Recorded Previous No-Shows can refine the groups selected for testing where history quality is sufficient. The validated MONITOR → DIAGNOSE → PRIORITIZE dashboard connects operational monitoring, interpretation of the analytical evidence and group-level decision support; its diagnostic and prioritisation views refer to the validated reference population.

Recommended action and evaluation

Test targeted reminders, confirmations or other attendance-support approaches in a predefined population and period before broader deployment. Monitor the No-Show Rate in the evaluated longer-lead group and, where appropriate, by recorded previous no-show status. Compare results against a reference defined before the test, then extend, adjust or stop the approach on the basis of the measured outcome.

Interpretation and limitations

The findings are observational associations, not evidence that lead time or reminders cause or prevent no-shows. Reminder Coverage is 72.68% of all 5,000 appointments and measures recorded reminder sending, not effectiveness; the observed reminder-status difference remains secondary. Recorded previous no-shows is an appointment-level field and does not establish a reliable longitudinal patient history. Distance is also secondary for standalone Analytics prioritisation. The proposed prioritisation identifies groups for intervention testing; it is neither an individual risk score nor evidence that an intervention has already reduced missed appointments.